

hovid-Amoxycillin Tablet

VIAM001-PC

DESCRIPTION

- **hovid-Amoxycillin Tablet 125mg**
Round, 8.0mm in diameter, pink uncoated tablet with tuttifrutti flavour, bevel-edged, flat faces, "HOVID" embossed on one face and scored on the other face.
- **hovid-Amoxycillin Tablet 250mg**
Round, 10.0 mm in diameter, light yellow uncoated tablet with grenadine flavour, bevel-edged, flat faces, "HOVID" embossed on one face and break-bar on the other.

COMPOSITION

- **hovid-Amoxycillin Tablet 125mg**
Amoxycillin (as trihydrate) 125 mg/tablet
- **hovid-Amoxycillin Tablet 250mg**
Amoxycillin (as trihydrate) 250 mg/tablet

ACTIONS AND PHARMACOLOGY

Amoxycillin, a bactericidal antibiotic, inhibits bacterial cell wall synthesis by preventing cross-linkage of peptidoglycan chains which is necessary for bacterial cell wall strength and rigidity. Cell division and growth are inhibited and lysis of susceptible bacteria frequently occurs.

Oral absorption is not greatly influenced by the presence of food and excretion is mainly via renal and biliary systems.

It is rapidly bactericidal and possesses the safety profile of a penicillin.

The wide range of organisms sensitive to the bactericidal action of Amoxycillin include:

- **Aerobes**
 - **Gram-positive:** *Streptococcus faecalis*, *Streptococcus pneumoniae*, *Streptococcus pyogenes*, *Streptococcus viridans*, *Staphylococcus aureus* (penicillin-sensitive strains only), *Corynebacterium species*: *Bacillus anthracis*, *Listeria monocytogenes*
 - **Gram-negative:** *Haemophilus influenzae*, *Escherichia coli*, *Proteus mirabilis*, *Salmonella species*, *Shigella species*, *Bordetella pertussis*, *Brucella species*, *Neisseria gonorrhoeae*, *Neisseria meningitidis*, *Vibrio cholerae*, *Pasteurella septic*
- **Anaerobes**
 - *Clostridium species*

PHARMACOKINETIC

Amoxycillin is well absorbed by the oral and parenteral routes. Oral administration, usually at convenient t.d.s. dosage, produces high serum levels independent of the time at which food is taken. Amoxycillin gives good penetration into bronchial secretions and high urinary concentrations of unchanged antibiotic.

In preterm infants with gestational age 26-33 weeks, the total body clearance after intravenous dosing of amoxycillin, day 3 of life, ranged between 0.75 – 2 ml/min, very similar to the inuline clearance (GFR) in this population. Following oral administration, the absorption pattern and the bioavailability of amoxycillin in small children may be different to that of adults. Consequently, due to the decreased CL, the exposure is expected to be elevated in this group of patients, although this increase in exposure may in part be diminished by decreased bioavailability when given orally.

INDICATIONS

For treatment of :

- Ear, nose and throat infections caused by *streptococci*, *pneumococci*, non-penicillinase - producing *staphylococci* and *H. influenzae*.
- Genitourinary tract infections caused by *E. coli*, *P.mirabilis*, *S. faecalis*.
- Skin and soft-tissues infections caused by *streptococci*, non-penicillinase - producing *staphylococci* and *E. coli*.
- Anogenital and urethral *gonorrhoea* caused by *N. gonorrhoea*.

CONTRAINDICATIONS

- Contraindicated in patients known to be sensitive to penicillin.
- Avoid in patients with infectious mononucleosis because of increased risk of skin rashes.

PRECAUTIONS

- As with any potent drug, periodic assessment of renal, hepatic and hematopoietic function should be made during prolonged therapy. The possibility of superinfections with mycotic or bacterial pathogens should be kept in mind during therapy. If superinfections occur (usually involving *Enterobacter*, *Pseudomonas* or *Candida*), the drug should be discontinued and/or appropriate therapy instituted.
- Caution in patients with known histories of allergy.
- Should be used with caution in pregnancy and lactation. Safety for use in pregnancy has not been established.
- Reduced doses may be required in patients with severe impaired renal function.

WARNINGS

Serious and occasionally fatal hypersensitivity (anaphylactoid) reactions have been reported in patients on penicillin therapy although anaphylaxis is more frequent following parenteral therapy, it has occurred in patients on oral penicillins. These reactions are more likely to occur in individuals with a history of sensitivity to multiple allergens.

There have been reports of individuals with a history of penicillin hypersensitivity who have experienced severe reactions when treated with Cephalosporins. Before therapy with any penicillin, careful inquiry should be made concerning previous hypersensitivity reactions to Penicillins, Cephalosporins or other allergens. If an allergic reaction occurs, appropriate therapy should be instituted and discontinuance of amoxicillin therapy considered. Serious anaphylactoid reactions require immediate emergency treatment with Epinephrine. Oxygen, intravenous steroids and airway management, including intubation, should also be administered as indicated.

PREGNANCY AND LACTATION

- **Use in pregnancy:** When antibiotic therapy is required during pregnancy, Amoxicillin may be considered appropriate when the potential benefits outweigh the potential risks associated with treatment.
- **Use in lactation:** Amoxicillin may be given during lactation. With the exception of the risk of sensitisation associated with the excretion of trace quantities of amoxicillin in breast milk, there are no known detrimental effects for the breast-fed infant.

SIDE/ADVERSE EFFECTS

- **Hypersensitivity reaction:** Rash and less frequently as exfoliative dermatitis or erythema multiforme.
- **Hemolytic effects:** Hemolytic anemia / anemia / eosinophilia / leukopenia / neutropenia / agranulocytosis / thrombocytopenia / thrombocytopenic purpura.
- **G.I. effects:** Diarrhoea, nausea, vomiting, black hairy tongue, glossitis, stomatitis, sore mouth or tongue.
- **Renal effects:** Acute interstitial nephritis.
- **Hepatic effects:** A moderate increase in serum concentration of AST (SGOT).

DRUG INTERACTIONS

- Concurrent use with Allopurinol or Probenecid requires careful monitoring.
- Concurrent use with Chloramphenicol, Erythromycins, Sulfonamides and Tetracycline may interfere with the bactericidal effect of amoxycillin.

OVERDOSE

Clinical features:

Anorexia, nausea, vomiting, abdominal discomfort, diarrhoea.

Treat overdose by emesis or gastric lavage, if appropriate and other supportive measures.

DOSAGE AND ADMINISTRATION

Adults

Oral, 250 to 500 mg every eight hours; or as directed.

Children

Infants up to 6 kg of body weight:

Oral, 25 - 50 mg every 8 hours.

Infants 6 - 8 kg of body weight:

Oral, 50 - 100 mg every 8 hours.

Infants and children 8 - 20 kg of body weight:

Oral, 6.7 - 13.3 mg/kg of body weight every 8 hours.

Children 20 kg of body weight and over:

See adult dose.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage:

Store below 25°C. Protect from moisture.

Presentation/Packing:

Tablet 125 mg x 100's, 1000's, Blisters 10 x 10's
Tablet 250 mg x 100's, 500's, Blisters 10 x 10's

Manufactured by: HOVID Bhd.
121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

Revision date: August 2015